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YOUR TURNING 65 WORKBOOK

The Turning 65 Medicare Decision Kit

Organize your deadlines, doctors, prescriptions and likely costs before you choose coverage.

ENROLLMENT TIMELINE

COVERAGE COMPARISON

COST WORKSHEET

DOCTOR + DRUG LISTS

PERSONAL ACTION PLAN

How to use this kit

Complete the worksheets before you compare plans or speak with an agent. Bring the finished kit to the conversation so your options can be checked against your real healthcare needs.

UPDATED JULY 2026

Educational resource | Printable and fillable

Start here: your Medicare snapshot

Check every statement that applies. These details can change when you should enroll and which coverage tradeoffs matter most.

Which statements describe you?

- | | |
|---|--|
| ☐ I will turn 65 within the next 12 months. | ☐ I or my employer contributes to an HSA. |
| ☐ I already receive Social Security or RRB benefits. | ☐ I have COBRA, retiree, VA, TRICARE or Marketplace coverage. |
| ☐ I or my spouse will keep working after I turn 65. | ☐ I take one or more prescription medications. |
| ☐ I currently have employer group health coverage. | ☐ Keeping specific doctors or hospitals is important. |

Medicare at a glance

PART A**Hospital insurance**

Inpatient hospital care, skilled nursing facility care after a qualifying stay, hospice and limited home health care.

PART B**Medical insurance**

Doctor visits, outpatient care, preventive services, durable medical equipment and other medically necessary services.

PART C**Medicare Advantage**

A private-plan way to receive Part A and Part B benefits. Most plans include Part D and may offer extra benefits.

PART D**Prescription drug coverage**

Private insurance that helps cover prescriptions. It can be included in Medicare Advantage or purchased separately.

2026 quick costs: standard Part B premium \$202.90/month; Part B deductible \$283; Part A inpatient deductible \$1,736 per benefit period; Part D covered-drug out-of-pocket cap \$2,100. [8][9]

Your turning 65 timeline

Plan early so your coverage is settled before the month Medicare should begin.



12 MONTHS BEFORE

Learn the basic paths

Review Parts A, B, C and D. Decide whether keeping certain doctors, traveling, monthly predictability or a low premium matters most.

6 MONTHS BEFORE

Check work coverage and HSA rules

Ask your benefits administrator which plan pays first, whether your drug coverage is creditable and what happens to dependents. Review HSA contribution timing before enrolling. [3][10]

3 MONTHS BEFORE

Your usual enrollment window opens

The Initial Enrollment Period generally begins three months before the month you turn 65. Apply early if you want coverage ready for your eligibility month. [1][4]

BIRTHDAY MONTH

Confirm every effective date

Verify Part A, Part B, drug coverage and any supplemental coverage separately. Do not assume all coverage starts on the same date. [2]

1 TO 3 MONTHS AFTER

Use the rest of your IEP carefully

The Initial Enrollment Period generally ends three months after your birthday month. Enrolling later can delay the start of coverage. [1][2]

Birthday on the first day of a month? Medicare generally treats your coverage timing differently, with premium-free Part A beginning the month before you turn 65. Confirm your exact dates with Social Security. [2]

Which enrollment path fits you?

These questions identify situations that need extra checking. They do not replace confirmation from Social Security, Medicare or your benefits administrator.

Do not make an enrollment decision based on employer size alone. Ask which coverage pays first and whether you need Part A and Part B for the employer plan to pay correctly. [3][10]

1

Already receiving Social Security?

Many people receiving benefits before 65 are enrolled automatically. Check your Medicare card or online status instead of assuming.

☐ This applies to me

2

Covered through active employment?

Coverage based on your or your spouse's current job may support delayed Part B, but the rules depend on the plan and employment status.

☐ This applies to me

3

Employer has 20 or more employees?

For many workers age 65 or older, a 20-plus employee group plan pays first. Smaller-employer coverage may pay second. Confirm in writing. [10]

☐ This applies to me

4

Contributing to an HSA?

Medicare enrollment can affect HSA eligibility. Premium-free Part A may be retroactive when enrollment occurs after 65. Get tax guidance before contributing. [3][4]

☐ This applies to me

5

Using COBRA or retiree coverage?

COBRA does not extend the Part B Special Enrollment Period tied to active employment. Retiree coverage often pays after Medicare. [1]

☐ This applies to me

6

Using VA, TRICARE or Marketplace coverage?

Each works differently with Medicare. Confirm the exact enrollment and coordination rules for your coverage before making changes.

☐ This applies to me

Ask your benefits administrator

☐ Which coverage pays first after I turn 65?

☐ Is the prescription coverage creditable for Part D?

☐ Will my spouse or dependents lose coverage if I enroll?

☐ What documentation should I keep for a later Special Enrollment Period?

Write down your key dates

A deadline you can see is much harder to miss. Confirm these entries using Medicare.gov or Social Security before relying on them.

DATE OF BIRTH**MONTH YOU TURN 65****IEP BEGINS****IEP ENDS****CURRENT COVERAGE ENDS****PLANNED RETIREMENT DATE****DESIRED MEDICARE START****CONFIRMED EFFECTIVE DATE**

Your Initial Enrollment Period generally lasts seven months: the three months before your birthday month, the birthday month and the three months after it. [1]

1**PART B AFTER ACTIVE EMPLOYMENT**

A common Special Enrollment Period lasts eight months after employment or qualifying group coverage ends, whichever happens first. COBRA does not stop this clock. [1]

2**MEDIGAP OPEN ENROLLMENT**

The federal six-month Medigap window starts the first month you are 65 or older and have Part B. After it, options may be limited or cost more. [6]

3**PART D CREDITABLE COVERAGE**

Going 63 days or more without Part D or other creditable prescription coverage can trigger a late penalty when you enroll later. [7]

Compare your two main coverage paths

Compare access, costs, prescriptions, travel and plan rules before choosing a path.

FACTOR	ORIGINAL MEDICARE + optional Medigap + Part D	MEDICARE ADVANTAGE private plan for Part A + Part B
Doctors	Any U.S. doctor or hospital that accepts Medicare.	Often uses a provider network and service area.
Specialists	Usually no referral is needed.	Some plans require referrals.
Drug coverage	Usually add a separate Part D plan.	Most plans include Part D coverage.
Cost structure	No annual out-of-pocket limit without supplemental coverage.	Has an annual limit for covered Part A and B services.
Extra benefits	Routine dental, vision and hearing usually are not included.	May include extra dental, vision, hearing or other benefits.
Approvals	Prior authorization is generally less common.	Prior authorization may be required for certain care.
Travel	Broad U.S. access where Medicare is accepted.	Routine care may be limited outside the service area.

Choose your top three priorities

- ☐ Broad provider access
 ☐ Low monthly premium
- ☐ Predictable healthcare spending
 ☐ Prescription fit
- ☐ Travel flexibility
 ☐ Dental, vision or hearing extras

Comparison summarized from Medicare.gov. Plan rules, benefits, networks and costs vary. [5]

Your doctors and care inventory

Do not rely only on a directory. Confirm participation with both the plan and the provider, then

Doctor or provider	Specialty	City	Medicare / network status	Checked on

Other care preferences

PREFERRED HOSPITAL OR HEALTH SYSTEM

PREFERRED PHARMACY

EQUIPMENT OR SUPPLY PROVIDER

REHAB OR SKILLED NURSING FACILITY

UPCOMING TESTS, PROCEDURES OR ONGOING TREATMENT

Your prescription inventory

Use the exact drug name, dose and frequency. A plan that covers one strength or form may

Drug name	Dose	How often	Pharmacy	Tier / cost / notes

Check each plan for

- ☐ Every drug is on the formulary
- ☐ Your pharmacy is preferred or in network
- ☐ Prior authorization requirements
- ☐ Step therapy requirements
- ☐ Quantity limits
- ☐ Mail-order rules and prices

For 2026, annual out-of-pocket spending on covered Part D drugs is capped at \$2,100. Premiums, non-covered drugs and certain other costs do not count toward that cap. [9]

QUESTIONS TO ASK THE PLAN OR AGENT

Compare your true Medicare costs

Compare premiums, expected care costs and high-use-year exposure.

ANNUAL COST CATEGORY	OPTION A Original + supplement + Part D	OPTION B Medicare Advantage
Part B premium (12 months)		
Plan / Medigap premiums		
Part D or drug-plan premiums		
Expected prescription costs		
Expected visits and copays		
Dental, vision and hearing		
Travel / out-of-network exposure		
Estimated annual total		

High-use-year reality check

OPTION A: LIKELY MAXIMUM EXPOSURE

OPTION B: PLAN OUT-OF-POCKET MAXIMUM

Original Medicare has no annual out-of-pocket limit unless you have other coverage, such as Medigap, Medicaid or employer coverage. Medicare Advantage plans have annual limits for covered Part A and Part B services. [5]

Planning formula: annual premiums + likely prescriptions + routine cost sharing + a realistic amount for unexpected care. Compare the result with your emergency savings and monthly budget.

Seven expensive Medicare mistakes

Most Medicare problems start with one bad assumption. Use this list as a final check before you enroll or delay coverage.

1 Treating COBRA like active-employer coverage

WHY IT MATTERS

The Part B Special Enrollment Period generally starts when active employment ends, even if COBRA continues.

DO THIS Record the employment end date and confirm your Part B deadline. [1]

2 Ignoring the HSA and Medicare timing conflict

WHY IT MATTERS

Medicare enrollment can make HSA contributions ineligible, and Part A can be retroactive when you enroll after 65.

DO THIS Coordinate Medicare, Social Security and HSA dates with qualified tax guidance. [3][4]

3 Choosing a plan because the premium is \$0

WHY IT MATTERS

Premiums are only one cost. Copays, coinsurance, drug costs, networks and the annual maximum can matter more.

DO THIS Complete the true-cost worksheet on page 9 before comparing plans.

4 Checking doctors but forgetting prescriptions

WHY IT MATTERS

A plan can fit your doctors and still make an important drug expensive or subject to restrictions.

DO THIS Check every drug, dose, pharmacy, tier and coverage rule.

Finish the mistake check

Then use the questions below to make sure a plan comparison addresses the issues that actually affect you.

5

Assuming Medicare enrollment is automatic

WHY IT MATTERS

Automatic enrollment depends on your situation. Waiting for a card that is not coming can create a coverage gap.

DO THIS

Verify your enrollment status with Social Security. [4]

6

Missing the one-time Medigap window

WHY IT MATTERS

After the federal six-month open enrollment period, insurers may be able to use medical underwriting in many situations.

DO THIS

Write down the month your Part B coverage starts. [6]

7

Dropping other coverage before confirming new dates

WHY IT MATTERS

Part A, Part B, Part D and private coverage can have different effective dates.

DO THIS

Keep proof of every effective date before canceling current coverage.

Questions to ask a licensed agent

- | | |
|---|---|
| ☐ Are all my doctors and hospitals covered? | ☐ Could I face underwriting if I switch later? |
| ☐ Are all my prescriptions on the formulary? | ☐ Which benefits can change each year? |
| ☐ What needs prior authorization? | ☐ Are my pharmacies preferred or in network? |
| ☐ What is my realistic high-use-year cost? | ☐ How does the plan handle specialist care? |
| ☐ What happens when I travel or move? | ☐ What enrollment deadline applies to me? |

Your one-page Medicare action plan

Turn the research into a short list of decisions, owners and dates. Keep a copy of every confirmation you receive.

MY PRIMARY COVERAGE GOAL

MY TARGET DECISION DATE

COVERAGE PATHS I WILL COMPARE

AGENT / BENEFITS APPOINTMENT

NEXT ACTION	OWNER	DUE DATE	DONE
			☐
			☐
			☐
			☐
			☐
			☐

NOTES AND CONFIRMATIONS

Keep copies of enrollment confirmations, employer coverage records, creditable drug coverage notices and plan effective dates.

Plain-English Medicare glossary

These are the terms most likely to affect enrollment, access or cost. Keep this page nearby while comparing coverage.

Assignment

An agreement by a provider to accept the Medicare-approved amount as full payment for a covered service.

IRMAA

An income-related surcharge that some higher-income beneficiaries pay for Part B and Part D.

Coinsurance

A percentage of the approved cost you pay after any deductible is met.

Maximum out-of-pocket

The most a Medicare Advantage plan requires you to pay in a year for covered Part A and Part B services.

Copayment

A fixed amount you pay for a covered service, visit or prescription.

Medigap

Private insurance that works with Original Medicare to help pay certain deductibles, copays and coinsurance.

Creditable drug coverage

Prescription coverage expected to pay, on average, at least as much as standard Medicare drug coverage.

Network

The providers and facilities contracted with a plan to deliver care at specified rates.

Deductible

The amount you pay before a plan begins sharing certain covered costs.

Prior authorization

Approval a plan may require before it covers certain services, supplies or drugs.

Formulary

A plan's list of covered prescription drugs, often organized into pricing tiers.

Service area

The geographic area where a health plan accepts members and usually provides routine care.

Guaranteed issue right

A protection requiring an insurer to sell you certain Medigap policies in qualifying situations.

Special Enrollment Period

A limited period to enroll or change coverage after a qualifying event or circumstance.

If a term appears in a plan document and you are unsure what it means for your costs or access, ask for a plain-language example using your own doctors and prescriptions.

Official sources and your next step

Medicare rules and plan details can change. Use these official pages to verify information before making a coverage decision.

- [1] [Medicare.gov: When can I sign up for Medicare?](#)
- [2] [Medicare.gov: When does Medicare coverage start?](#)
- [3] [Medicare.gov: Working past 65](#)
- [4] [Social Security: When to sign up for Medicare](#)
- [5] [Medicare.gov: Compare Original Medicare and Medicare Advantage](#)
- [6] [Medicare.gov: Get ready to buy a Medigap policy](#)
- [7] [Medicare.gov: Part D late enrollment penalty](#)
- [8] [CMS: 2026 Medicare Parts A and B premiums and deductibles](#)
- [9] [CMS: Final CY 2026 Part D redesign instructions](#)
- [10] [Medicare.gov: Who pays first?](#)

Ready to compare your options?

A licensed Medicare agent can review your completed workbook and compare coverage against your doctors, prescriptions, budget and

[MEDICAREFAQ.COM/MEDICARE-PLANS](https://www.MedicareFAQ.com/Medicare-Plans)

(888) 335-8996



IMPORTANT DISCLOSURE

This workbook is for educational purposes only. It is not legal, tax or medical advice and does not determine eligibility, benefits or plan availability. Confirm dates and coverage with Medicare, Social Security, your employer benefits administrator or a licensed insurance agent. Plan benefits, costs, formularies and networks vary and may change.

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